Clinical Practice Guideline for Prescribing Opioids for Pain: Quick Reference Guide

This resource has been adapted from the CDC Clinical Practice Guideline for Prescribing Opioids for Pain — United States, 2022. The CDC recommends that persons with pain receive appropriate pain treatment, with careful consideration of the benefits and risks of all treatment options in the context of the patient's circumstances. Refer to the guiding principles, which support adapting these recommendations to individual patient needs rather than applying them as inflexible standards of care across patient populations. The 2022 CDC Guideline excludes pain management related to sickle cell disease, cancer-related pain treatment, palliative care, and end-of-life care. It also clarifies that cancer-related pain treatment applies to cancer survivors. In addition, these guidelines are not comprehensive regarding adjustments needed for women's health conditions or pediatric patients.

Key Area	Recommendations
Determining Whether or Not to Initiate Opioids for Pain	***Recommendations 1 and 2*** **Before prescribing opioids for acute pain** • Try nonpharmacologic and nonopioid therapies as appropriate for the specific condition and patient. • Nonopioid therapies are at least as effective as opioids for acute pain. • Only consider opioids for acute pain if nonpharmacologic and nonopioid therapies are not expected to be effective and if the benefits are anticipated to outweigh the risks. • Discuss the realistic benefits and known risks of opioid therapy with patients. **Before prescribing opioids for subacute or chronic pain** • Try nonpharmacologic and nonopioid therapies as appropriate for the specific condition and patient. • Discuss the realistic benefits and known risks of opioid therapy with patients. • Set treatment goals for pain and function, and plan how to discontinue opioids if the benefits do not outweigh the risks.
Selecting Opioids and Determining Dosages	***Recommendations 3-5*** **When starting opioid therapy for acute, subacute, or chronic pain** • Prescribe immediate-release opioids instead of extended-release and long-acting (ER/LA) opioids. • Prescribe the lowest effective dosage of opioids for opioid-naïve patients. • When selecting opioids, clinicians must consider the patient's renal function. • Clinicians should establish an expected duration and response from opioids, as well as elicit and document the patient's goals and expectations.

Published on: April 18, 2025

© 2025 American Medical Association

	If opioids are continued for subacute or chronic pain • Use caution when prescribing opioids at any dosage. • Carefully evaluate individual benefits and risks when considering increasing dosage. • Avoid increasing dosage above levels likely to yield diminishing returns in benefits relative to risks to patients. **For patients already receiving opioid therapy** • Carefully weigh the benefits and risks and exercise care when changing opioid dosage. • If the benefits outweigh the risks of continued opioid therapy, work closely with patient to optimize nonopioid therapies. • If the benefits do not outweigh the risks of continued opioid therapy, optimize other therapies and work closely with the patient to gradually taper to lower dosages. • Do not rapidly reduce opioid dosages from higher dosages. • Do not discontinue opioid therapy abruptly, unless there are indications of a life-threatening issue such as warning signs of impending overdose (e.g., confusion, sedation, or slurred speech).
Deciding Duration of Initial Opioid Prescription and Conducting Follow-up	***Recommendations 6 and 7*** **When opioids are needed for acute pain** • Prescribe no greater quantity than needed for the expected duration of pain. • Evaluate the benefits and risks with patients within 1–4 weeks of starting opioid therapy for subacute or chronic pain or of dosage escalation. • Regularly re-evaluate the benefits and risks of continued opioid therapy with patients.
Assessing Risk and Addressing Potential Harms of Opioid Use	***Recommendations 8-12*** **Before starting and periodically during continuation of opioid therapy** • Evaluate risk for opioid-related harms and discuss these risks with patients, including medical risks such as delirium, falls, sedation, respiratory insufficiency, renal insufficiency, etc. • Use validated tools or consult with behavioral specialists to screen for and assess mental health and substance use disorders. • Check the PDMP before prescribing opioids and periodically during long-term opioid therapy to determine whether the patient is receiving opioid dosages or combinations that put the patient at high risk for overdose.

Published on: April 18, 2025
© 2025 American Medical Association

	• Work with patients to incorporate strategies to mitigate risk, including prescribing naloxone, especially for patients at increased risk for overdose, such as those with a history of overdose, substance use disorder, or those taking higher dosages of opioids or concurrent benzodiazepines. **When prescribing initial opioid therapy for acute, subacute, or chronic pain, and periodically during opioid therapy for chronic pain** • Review the patient's history of controlled substance prescriptions using state prescription drug monitoring program (PDMP) data to determine whether the patient is receiving opioid dosages or combinations that put the patient at high risk for overdose. **When prescribing opioids for subacute or chronic pain** • Consider the benefits and risks of toxicology testing to assess prescribed medications as well as other prescribed and nonprescribed controlled substances. • Use caution when prescribing opioid pain medication and benzodiazepines concurrently and consider whether the benefits outweigh the risks of concurrent prescribing of opioids and other central nervous system depressants. • Review any other medications the patient is taking to avoid potential interactions with opioids. **For persons with opioid use disorder** • Offer or arrange treatment with evidence-based medications to treat patients with opioid use disorder. • Co-treatment of pain using opioid analgesics for appropriate indications alongside evidence-based medications for OUD (buprenorphine or methadone), in collaboration with the addiction clinician, is appropriate. • Detoxification on its own, without medications for opioid use disorder is not recommended for opioid use disorder because of increased risks. • Prescribing naloxone to people with OUD is strongly recommended.

Guiding Principles for Implementation

1. Assess and treat acute, subacute, and chronic pain appropriately, regardless of whether opioids are part of a treatment regimen.
2. Recommendations are to support individualized, person-centered care, ensuring flexibility for specific patient needs.
3. Use a multimodal, multidisciplinary approach to pain management, addressing physical and behavioral health, long-term support, and overall well-being.
4. Avoid misapplying guidelines or implementing related policies that may harm patients.
5. Address health inequities, provide culturally and linguistically appropriate communication, ensure accessibility for disabilities, and offer diverse, coordinated pain management options.

Published on: April 18, 2025
© 2025 American Medical Association